ISSUED, VALID FROM 30 SEPTEMBER 2026

Tetanus, Diphtheria and Polio (Revaxis, Td/IPV)

Patient Group Direction, version 011, issued 30 September 2026. Revaxis, individuals aged 10 years and over.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Key rules in this version

- **PREGNANCY IS AN UNQUALIFIED EXCLUSION. Refer every pregnant individual to the GP or midwife, including after a tetanus-prone wound, the same day. This is a Get Real Health service decision that departs from Green Book chapter 30, which permits tetanus-containing vaccines in pregnancy without delay; the same-day referral is what stops the departure delaying a needed dose.**
- **A TETANUS-PRONE WOUND IS ASSESSED BY TIME SINCE THE LAST DOSE, NOT BY DOSE COUNT. Adequate priming (3 or more doses) with the last dose within 10 years: no vaccine. Last dose more than 10 years ago, whatever the total number of doses: a reinforcing dose under this PGD. High-risk wound: immunoglobulin as well, which is a same-day referral (UKHSA Tetanus: advice for health professionals, Table 4).**
- **A DOSE WITHIN THE LAST 12 MONTHS EXCLUDES, except the second or third dose of a primary course being given under this PGD at the scheduled one-month interval.**
- **Observe every patient for 15 minutes after vaccination.**

Revaxis (Td/IPV)

Summary of Green Book chapter 30 guidance for tetanus

UKHSA, Immunisation Against Infectious Disease, chapter 30, Tetanus. Chapter dated 2 June 2025; GOV.UK page last updated 14 August 2026. Summarised 9 September 2026.

Overview

Tetanus is caused by tetanus toxin released after infection with Clostridium tetani. Spores are present in soil and manure and enter through a puncture wound, burn or scratch that may go unnoticed.

Incubation is 4 to 21 days, most commonly about 10 days. It causes generalised rigidity and spasm of skeletal muscle, usually starting in the jaw and neck.

The case fatality ratio ranges from 10 to 90 per cent, highest in infants and the elderly.

It cannot be eradicated, because the spores are in the environment, and it is not spread person to person.

Between 1984 and 2022 there were 327 cases in England and Wales, 68.3 per cent in people aged 45 or over.

Who the chapter recommends it for

The objective of the programme is a minimum of FIVE doses of a tetanus-containing vaccine at appropriate intervals for all individuals.

Td/IPV is the vaccine recommended for everyone aged 10 years or over.

TRAVELLERS: all travellers should be fully immunised to the UK schedule. Where a traveller is going somewhere medical attention may not be accessible and their last dose was more than 10 years ago, a booster should be given before travel EVEN IF they have already had five doses. This is precautionary, in case immunoglobulin is not available if they sustain a tetanus-prone injury.

Where tetanus, diphtheria or polio protection is needed and the last relevant dose was over 10 years ago, give Td/IPV.

People who inject drugs are at greater risk; take every opportunity to ensure they are protected, and give a booster if there is any doubt.

Where there is no reliable history, assume undocumented doses are missing and follow the UK catch-up recommendations for that age.

Schedule

All tetanus-containing vaccines are single 0.5 mL doses.

Aged 10 or over, primary course: three doses of a vaccine containing at least 20 IU of tetanus toxoid, four weeks apart.

An interrupted course is RESUMED, not repeated, with four weeks between the remaining doses.

Second booster: Td/IPV, ideally 10 years after the first tetanus booster.

Someone aged 10 or over who has had only three doses should have the first booster as Td/IPV ideally five years after their last primary dose, and the second booster a minimum of five years after that.

If someone attends for a routine booster having had a vaccine after a tetanus-prone wound: if it was the same vaccine now due and given after an appropriate interval, the routine booster is not required. Otherwise discount the injury dose and give the scheduled immunisation. Additional doses are unlikely to produce an unacceptable rate of reactions.

Contraindications and precautions

Do not give to anyone who has had a confirmed anaphylactic reaction to a previous dose of a tetanus-containing vaccine, or to any component or manufacturing residue.

There are very few individuals who cannot receive tetanus-containing vaccines. Where in doubt, seek specialist advice rather than withholding the vaccine.

Minor illness without fever or systemic upset is not a reason to postpone. If acutely unwell, postpone until recovered.

Previous systemic or local reactions are NOT a bar to further doses, including fever of any severity, hypotonic-hyporesponsive episode, persistent crying over three hours, a severe local reaction of any extent, and convulsions within three days of vaccination.

Neurological conditions are not a contraindication, though deferral may be considered where there is current neurological deterioration.

Pregnancy and immunosuppression

PREGNANCY: tetanus-containing vaccines may be given to pregnant women without delay when protection is required. There is no evidence of risk from inactivated vaccines or toxoids in pregnancy or breastfeeding.

IMMUNOSUPPRESSION: give in accordance with the recommendations, including in HIV regardless of CD4 count. A full antibody response may not be made; consider re-immunisation after treatment finishes.

Severely immunosuppressed people may not be adequately protected despite full immunisation, and may need additional boosting or immunoglobulin after exposure.

Two things to know about this chapter

UKHSA states that chapter 30 IS BEING UPDATED for immunoglobulin and post-exposure advice, and directs readers to the separate guidance Tetanus: advice for health professionals in the interim. Anything about tetanus-prone wound management should follow that interim guidance rather than the chapter alone.

Patient Group Direction for the administration of Revaxis, adsorbed diphtheria (low dose), tetanus and inactivated poliomyelitis vaccine, to individuals aged 10 years and over.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- All users must be trained to the National Minimum Standards and Core Curriculum for Immunisation Training.
- All users must be competent in the recognition and immediate management of anaphylaxis, and in vaccine handling and cold chain management.
- All users must be able to apply UKHSA Tetanus: advice for health professionals (Table 4), which governs tetanus-prone wound management while Green Book chapter 30 is updated, and must know that a decision about tetanus immunoglobulin is a referral and not a PGD supply.
- All users must be able to count documented doses and to recognise when five have been given.
- All users must be familiar with Gillick competence and the assessment of consent in children and young people, and with the involvement of a person with parental responsibility.
- All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required, must work in compliance with the SOPs of their own employer, and must hold appropriate indemnity covering this service.
- All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.
- All users must be up to date with CPD and appraisal, and with MHRA safety alerts relating to this product.
- All users must understand capacity and consent and be aware of the Mental Capacity Act 2005.

The PGD

Indication	Active immunisation of individuals aged 10 years and over against tetanus, diphtheria and poliomyelitis, in accordance with chapters 15, 26 and 30 of Immunisation Against Infectious Disease (the Green Book). This PGD supports: the routine adolescent booster where it has been missed; completion of a primary course or booster where the immunisation history is incomplete, unknown or out of date; protection for travel in accordance with NaTHNaC recommendations; and a reinforcing dose for a tetanus-prone wound where immunoglobulin is not indicated.
Inclusion criteria	● Individuals aged 10 years and over. ● Valid informed consent obtained from the individual, or from a person with parental responsibility where the individual is under 16 and not Gillick competent. ● **No tetanus, diphtheria or polio containing vaccine received in the last 12 months, unless the dose being given is the second or third dose of a primary course under this PGD at the scheduled one-month interval.** ● **AND ONE OF THE FOLLOWING:** ○ Requires a booster following a primary course against diphtheria, tetanus and poliomyelitis, usually offered at 13 to 18 years of age where not already completed; OR ○ Has no history, or an incomplete or uncertain history, of diphtheria, tetanus or poliomyelitis immunisation; OR ○ Is travelling to an area where medical attention may not be accessible should a tetanus-prone wound occur, or will be residing in an epidemic or endemic area, AND the final dose of the relevant antigen was received more than 10 years ago.

	This applies even where 5 doses have previously been received; OR ○ **Has a tetanus-prone wound where a reinforcing dose is indicated under UKHSA Tetanus: advice for health professionals (Table 4): an adequate priming course (3 or more doses) with the last dose more than 10 years ago, whatever the total number of doses, or an incomplete or uncertain history. Where the wound is high risk the dose is still given and the patient is referred the same day for immunoglobulin as well.** ● No exclusion criterion present.
Five documented doses: what it means, and where it does not apply	**Five documented doses of a tetanus-containing vaccine is a complete lifetime course for ROUTINE purposes: no further scheduled boosters are due. It does NOT exempt the individual from a wound dose or a travel booster when the last dose was more than 10 years ago, because UKHSA's wound guidance and the Green Book travel advice both go by time since the last dose.** **TETANUS-PRONE WOUND: adequate priming and the last dose within 10 years, no vaccine. Last dose more than 10 years ago, give a reinforcing dose under this PGD, whatever the total number of doses.** ● **Where the wound is HIGH RISK (heavy contamination with soil or manure, devitalised tissue, burns, sepsis, or a delay of more than 6 hours to surgical treatment) tetanus immunoglobulin is indicated in addition to any vaccine dose. Immunoglobulin is not a PGD supply: give the vaccine dose where indicated and refer the same day for immunoglobulin.** ● **Where the person is NOT adequately primed (fewer than 3 documented doses of a primary course) or the vaccination history is uncertain, and the wound is tetanus-prone: give the vaccine dose under this PGD AND refer the same day for tetanus immunoglobulin, whether or not the wound is high risk. Where the person is adequately primed but the last dose was more than 10 years ago and the wound is HIGH RISK: give the reinforcing dose AND refer the same day for immunoglobulin. This follows UKHSA Tetanus: advice for health professionals (Table 4), which governs wound management while Green Book chapter 30 is updated.** ● Where the wound is not tetanus-prone, or the person is adequately primed with the last dose within 10 years, no immunisation action is needed. Give wound care advice and record the assessment. **TRAVEL, 5 doses already given: a booster IS indicated where the last dose was more than 10 years ago, and may be given under this PGD.** ● This is the one situation in which a fully immunised individual is boosted, and it exists because the traveller may not be able to reach medical care after a wound.
Exclusion criteria	**Refer, do not vaccinate, where any of the following applies.** ● **PREGNANCY, in all circumstances, including after a tetanus-prone wound. Refer to the GP or midwife THE SAME DAY. This is a Get Real Health service decision that departs from Green Book chapter 30, which permits tetanus-containing vaccines in pregnancy without delay; the same-day referral is what keeps the dose from being delayed. From week 16 of pregnancy a pertussis-containing vaccine is routinely indicated instead, which is a different product and a different decision.** ● **A tetanus, diphtheria or polio containing vaccine received within the last 12 months, EXCEPT where the dose is the second or third dose of a primary course being given under this PGD at the**

	scheduled one-month interval. Otherwise refer to establish what was given and when. ● Aged under 10 years. Refer to the GP or an appropriate immunisation service for an age-appropriate vaccine (DTaP/IPV/Hib/HepB or dTaP/IPV). ● Confirmed anaphylactic reaction to a previous dose of a diphtheria, tetanus or poliomyelitis containing vaccine, including any conjugate vaccine in which diphtheria or tetanus toxoid is used as the carrier. ● Confirmed anaphylactic reaction to any component of the vaccine, including neomycin, streptomycin or polymyxin B. ● Acute severe febrile illness. The presence of a minor infection is not a contraindication. ● **A tetanus-prone wound needing immunoglobulin is NOT an exclusion from the vaccine dose: give the dose where indicated and refer the same day for immunoglobulin, which this PGD does not supply.** ● **A tetanus-prone wound in an individual with an adequate priming course whose last tetanus-containing dose was within the last 10 years. No further vaccine is needed; where the wound is high risk, refer for immunoglobulin.** ● Management of cases or contacts in an outbreak of diphtheria or poliomyelitis. These individuals must be managed by the local Health Protection Team and are outside this PGD. ● Valid informed consent not given, or consent from a person with parental responsibility not obtained where required. ● Current neurological deterioration. Defer and seek advice so that any change is not incorrectly attributed to the vaccine. ● Neurological complications, including Guillain-Barre syndrome or brachial neuritis, following a previous diphtheria or tetanus containing vaccine. Refer.
Cautions	**Facilities and trained staff for the management of anaphylaxis must be available, with immediate access to adrenaline (epinephrine) 1 in 1,000 injection and a telephone (Green Book chapter 8 and Resuscitation Council UK guidance).** **OBSERVE EVERY PATIENT FOR 15 MINUTES AFTER VACCINATION. Vaccinate seated. Syncope can occur before or after vaccination, particularly in adolescents, and this PGD starts at 10 years of age. Ensure procedures are in place to avoid injury from a faint.** A stable neurological condition is not a contraindication. Only current deterioration defers. Immunosuppression: the immune response may be reduced. Vaccination is still recommended and the individual should be advised that protection may be limited. Revaxis contains approximately 10 micrograms of phenylalanine per 0.5 mL dose. The NSPKU advises this amount is negligible and that individuals with phenylketonuria should take up the offer of immunisation. Bleeding disorders and anticoagulation: see route and method of administration. **Where a person attends for a routine booster and has previously received a vaccine after a tetanus-prone wound, establish which vaccine was given and when. If it was the same vaccine and given after an appropriate interval, the routine booster is not required. If it was given within the last 12 months, this PGD excludes.**
Actions if the patient is excluded or declines	Advise on alternative options and how to access them, including the GP practice and, for travel, an appropriate travel health service. Explain the protective effect of the vaccine, the risk of infection and the potential

	complications of disease. Where a tetanus-prone wound requires immunoglobulin or medical assessment, arrange same-day referral and do not delay. Where the individual is pregnant, refer to the GP or midwife and make the urgency explicit if a wound is involved. Document the advice given and the decision reached. Inform or refer to the GP as appropriate.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	• That valid informed consent was given, and from whom. Where the individual is under 16, the name and relationship of the person with parental responsibility, or the basis of the Gillick assessment. • Patient name, address, date of birth, and the GP with whom they are registered. • **THE NUMBER OF DOCUMENTED PRIOR DOSES, and the source of that information.** • **THE DATE OF THE MOST RECENT TETANUS-CONTAINING DOSE, and how it was established.** • **Which indication applied: adolescent booster, incomplete history, travel, or tetanus-prone wound.** • **Where a wound is involved, the assessment against UKHSA Tetanus: advice for health professionals (Table 4) and the conclusion on immunoglobulin.** • Name and brand of vaccine, batch number and expiry date. • Date of administration, dose, route and anatomical site. • **That the 15 minute observation period was completed.** • Where off-label, that this was explained and consent given on that basis. • Name and registration number of the healthcare professional administering. • Advice given, including advice given if the individual is excluded or declines. • Details of any adverse drug reactions and the actions taken. • That the vaccine was administered under this PGD. Records signed, dated, legible and contemporaneous. Adults 18 and over: 8 years. Under 18s: until the 25th birthday, or the 26th where the individual was 17 when the course finished. Inform the GP.

The medicine

Name, form and strength	Revaxis, adsorbed diphtheria (low dose), tetanus and inactivated poliomyelitis vaccine, suspension for injection in a pre-filled syringe, 0.5 mL.
Legal category	POM.
Black triangle	No.
Off-label use	• Primary immunisation in individuals over 10 years of age is off-label but is administered in accordance with chapters 15, 26 and 30 of the Green Book. • **Where a vaccine is administered off-label, inform the individual, parent or carer as part of the consent process that it is being given outside its product licence but in accordance with national guidance, and record that you did.** • **Administration within 5 years of a previous diphtheria or tetanus toxoid containing vaccine is outside the SmPC (section 4.4) and is given in accordance with the Green Book. Explain this as part**

	of consent and record it. A dose within the last 12 months excludes outright.
Dose and frequency of administration	• Single 0.5 mL dose per administration. • Adolescent booster: a single 0.5 mL dose at around 14 years of age, a minimum of 5 years after the pre-school booster. • Previously unimmunised, or unknown or incomplete history: a primary course of 3 doses with an interval of one month between doses. Where a course is interrupted it should be resumed, not restarted. • First booster: at least 5 years after the third primary dose. Second booster: a minimum of 5 years and ideally 10 years after the first booster, where fewer than 5 documented doses have been given. • Travel: a single booster where the last dose was more than 10 years ago, including where 5 doses have already been given. • **Tetanus-prone wound: a single reinforcing dose where the last tetanus-containing dose was more than 10 years ago, whatever the total number of doses, or where the history is incomplete or uncertain, in accordance with UKHSA Tetanus: advice for health professionals.** • **In every case: no dose if a tetanus, diphtheria or polio containing vaccine has been given in the last 12 months, other than the scheduled second and third doses of a primary course under this PGD.**
Quantity to be administered	Single 0.5 mL dose per administration. This PGD does not permit supply of vaccine to the individual for administration elsewhere.
Route and method of administration	• Intramuscular injection, preferably into the deltoid muscle of the upper arm. • Shake the pre-filled syringe well before administration. The normal appearance is a cloudy white suspension that may sediment during storage. • Inspect visually for foreign particulate matter or any variation from the expected appearance. If present, do not administer and discard in accordance with local procedures. • Where given at the same time as other vaccines, use separate sites, preferably different limbs, or at least 2.5 cm apart in the same limb. Record the site of each. • Individuals on stable anticoagulation, including those on warfarin who are up to date with INR testing and whose latest INR was below the upper threshold of their therapeutic range, may be vaccinated intramuscularly using a 23 gauge or finer needle, followed by firm pressure without rubbing for at least 2 minutes. Advise on the risk of haematoma. Where the intramuscular route is not suitable, give by deep subcutaneous injection.
Storage and cold chain	Store at +2C to +8C. Do not freeze. Store in the original packaging to protect from light. Vaccine stored outside these conditions must be quarantined and risk assessed in accordance with UKHSA Vaccine Incident Guidance before any decision on continued use.
Disposal	Equipment used for immunisation, including used syringes and any discharged vaccine, must be disposed of in a UN-approved puncture-resistant sharps container in accordance with local arrangements and HTM 07-01.
Adverse effects	Very common and common: injection site pain, erythema and swelling, headache, malaise, myalgia, fatigue, fever. Uncommon: lymphadenopathy, nausea, dizziness. Rare: hypersensitivity reactions including urticaria and angioedema. Very rare: anaphylaxis, and extensive limb swelling which is self-limiting. Consult the current SPC.

Information for the patient

Written information	Supply the manufacturer's patient information leaflet, and a written record of the vaccine given with the date, brand and batch number. Tell the individual to keep it: the number of doses they have had determines what happens if they are ever injured.
Counselling	● A sore arm, mild fever, headache or aching for a day or two is common and settles by itself. ● **Keep the record of this vaccination. If you are ever injured, whether you need anything depends on how many doses you have had and when.** ● **For any dirty wound, a puncture wound, a burn, an animal bite, or a wound with soil or manure in it, clean it and seek medical advice, whatever vaccinations you have had.** ● Where a course is involved, come back for every dose. We have booked your next one. ● Where you are travelling: this protects you, but it does not remove the need to get any significant wound cleaned and assessed while you are away.
Follow-up	Book the next dose of any course at this appointment. Seek medical advice for any tetanus-prone wound. For a routine query about the vaccine, contact the pharmacy.

Appendix 1: The three questions to settle before vaccinating

1. Is the individual pregnant?	• **YES: EXCLUDE. Refer to the GP or midwife, same day where a wound is involved.** • This is now unqualified. There is no "except where protection is required without delay" route under this PGD.
2. Has any tetanus, diphtheria or polio containing vaccine been given in the last 12 months?	• **YES: EXCLUDE. Refer to establish what was given and when.** • NO: proceed to question 3. • Record the date and how it was established.
3. How many documented doses have they had?	• FEWER THAN 5: proceed on the applicable indication. • **5 OR MORE, and the reason is a WOUND: a reinforcing dose if the last dose was more than 10 years ago; none if within 10 years. Refer the same day for immunoglobulin in addition if the wound is high risk.** • **5 OR MORE, and the reason is TRAVEL with the last dose over 10 years ago: a booster is indicated and may be given.**

Appendix 2: Key references

- Immunisation Against Infectious Disease (the Green Book), chapter 15 (diphtheria), chapter 26 (poliomyelitis) and chapter 30 (tetanus), including table 30.1 on tetanus-prone wounds.
- Green Book chapter 8, immunisation procedures, for anaphylaxis provision.
- Summary of Product Characteristics for Revaxis, current version.
- NaTHNaC, travelhealthpro.org.uk, for travel recommendations.
- UKHSA Vaccine Incident Guidance.
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions, https://www.nice.org.uk/guidance/mpg2

Authorisation

Version	v011
Supersedes	Version 010, 14 September 2026
Valid from date	30 September 2026
Expiry date	31 July 2027

Authorised on behalf of Get Real Health by the Medical Director and the Head Pharmacist. Their signatures for this version are on the signed authorisation page at the end of this document.	

Adoption by the employing organisation

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....
Professional group signatory	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....

GET REAL HEALTH

Change history to version 004

Later versions are recorded in the Version and change record at the end of this document.

Version	v004
Date	8 September 2026
Changes	● The practitioner Agreement to practise page, the premises block and the practitioner signature table are restored. Every version 001 document carried them; no document produced by the current generator did, through one omission in one function, which left 15 live documents with no page for an individual practitioner to sign. Raised as blocking by an adopting pharmacy: NICE MPG2 expects a record of the individuals authorised to work under a PGD. The standard governance requirements are restored at the same time: SPC and BNF familiarity, MHRA safety alerts, CPD and appraisal, indemnity, and capacity and consent, all of which were in version 001 and were lost in the rewrite. This version changes no clinical content. ● Pregnancy is an unqualified exclusion in all circumstances, including after a tetanus-prone wound. Version 002 excluded pregnancy "except where protection is required without delay" in its exclusion criteria while its cautions said to refer pregnant individuals to the GP or midwife, so the same page both permitted and refused the supply. ● Five documented doses now handled explicitly, and differently for the two situations it arises in. For a tetanus-prone wound, 5 doses means no further vaccine is needed and the question becomes immunoglobulin, which is a referral. For travel with the last dose more than 10 years ago, a booster is still indicated. Version 002 stated only the travel case. ● A tetanus, diphtheria or polio containing vaccine given within the last 12 months is now an outright exclusion. Version 002 handled recent doses through an off-label note about the previous 5 years, which would have blocked a legitimate travel booster in someone who had a wound dose four years earlier. 12 months was chosen so that it does not. ● A 15 minute observation period after every vaccination is stated in the document, consistently with every other vaccination PGD in this estate. Version 002 noted the risk of syncope in adolescents without setting an observation period. ● Records must now carry the number of documented prior doses and its source, the date of the most recent tetanus-containing dose, which indication applied, and, where a wound is involved, the assessment against table 30.1 and the conclusion on immunoglobulin. ● Appendix 1 added: the three questions to settle before vaccinating, in order.

Previous versions

001	Development and issue of new PGD.
002, 6 August 2026	Revaxis Td/IPV, individuals aged 10 years and over, covering the adolescent booster, incomplete or unknown history, travel, and tetanus-prone wounds where immunoglobulin is not indicated. 003 to 007, September 2026: see the version and change record at the end of this document.

Standard requirements for every practitioner working under this PGD

All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.

All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.

All users must be up to date with their CPD requirements and appraisal.

All users must hold appropriate professional indemnity covering this service.

All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

Agreement to practise by the registered healthcare professional

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of the pharmacy or clinic premises to which this PGD relates

Premises name	.....
Address	.... Postcode: .....
ODS code, if applicable	.....

Registered healthcare professionals authorised to work under this PGD at those premises

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

Name of healthcare professional	Job title | Registration number | Signature | Date
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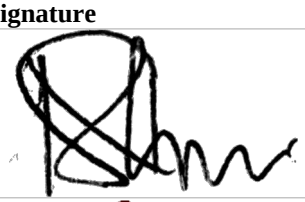
Version and change record
Version: v011
Issued: 30 September 2026
Supersedes: Version 010, 14 September 2026
Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records
Version: v010
Issued: 14 September 2026
Supersedes: Version 009, 11 September 2026
Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.
Previous version records
Version: v009
Issued: 11 September 2026
Supersedes: Version 008, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
The immunoglobulin rule now uses the same definition of adequate priming as the cover, the inclusion criteria, the dose row and Appendix 1: 3 or more documented primary doses (UKHSA Tetanus: advice for health professionals, Table 4). The previous wording said fewer than 5 documented doses, which contradicted the rest of the document.
The immunoglobulin rule, the training row and the records row now cite UKHSA Tetanus: advice for health professionals (Table 4), which part 2 identifies as governing wound management while Green Book chapter 30 is updated, instead of table 30.1.
The cover heading 'Three changes in this version', which sat above four bullets from earlier versions, now reads 'Key rules in this version'.
The change history table's previous versions list now points to the version and change record for versions 003 to 007.
Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.
Previous version records
Version: v008
Issued: 11 September 2026
Supersedes: Version 007, 10 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Immunoglobulin rule completed. The previous version indicated immunoglobulin only for high-risk wounds. Green Book chapter 30 also requires it for any tetanus-prone wound where the person is not adequately primed or the history is uncertain, and for a high-risk wound where the last dose was over 10 years ago. Both cases are now stated, with the vaccine dose given under this PGD and same-day referral for immunoglobulin.
Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual

customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.
Previous version records
Version: v005
Issued: 10 September 2026
Supersedes: Version 004, 9 September 2026
Change: the summary of the governing guidance is added as part 2 of the document, between the cover and the PGD, which is the house format for every Get Real Health PGD. It is summarised from the source it names and adds nothing to it. Nothing else in this document is altered, and this is not a clinical review of the remainder.
Version: v006
Issued: 10 September 2026
Supersedes: Version 005, 10 September 2026
Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded.
Exclusion added: neurological complications (including Guillain-Barre syndrome or brachial neuritis) following a previous diphtheria or tetanus containing vaccine, which the Revaxis SmPC contraindicates and the previous version did not mention.
The 12-month exclusion now carves out the second and third doses of a primary course given under this PGD at the scheduled one-month interval. As written it excluded the course the PGD says it supports.
Off-label statement restored: the Revaxis SmPC (4.4) still says the vaccine should not be given within 5 years of a previous diphtheria or tetanus toxoid containing vaccine. A dose between 12 months and 5 years is off-label and is given in accordance with the Green Book; this must be explained and recorded. Version narration removed from the cover and the off-label section.
Version: v007
Issued: 10 September 2026
Supersedes: Version 006, 10 September 2026
Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Tetanus-prone wound: assessed by time since the last dose, not by dose count. Version 006 said five documented doses meant no vaccine for a tetanus-prone wound, in the cover, the inclusion criteria, the exclusion criteria, the dose row and Appendix 1. UKHSA's Tetanus: advice for health professionals (Table 4), which this PGD's own part 2 says governs wound management, gives an immediate reinforcing dose to anyone with an adequate priming course whose last dose was more than 10 years ago, whatever the total. Every one of those places now says that. The Wound Care PGD is aligned in the same round.
Immunoglobulin: a wound needing immunoglobulin no longer excludes the vaccine dose. The dose is given under this PGD where indicated and the patient is referred the same day for immunoglobulin, which this PGD does not supply.
Pregnancy remains an unqualified exclusion. The document now states that this is a Get Real Health service decision that departs from Green Book chapter 30, which permits tetanus-containing vaccines in pregnancy without delay, and that the referral must be the same day so the dose is not delayed.

Signed on behalf of Get Real Health

Version v011, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.